IGNIS CARE LIMITED
30 Pipit Walk Hemel Hempstead HP3 0FR

Review Sheet

Last Reviewed 26 Nov 2025 | Last Amended 26 Nov 2025 | This policy will be reviewed as needs require or at the following interval: Annual

Business Impact:	LOW — Minimal action required. Circulate information amongst relevant parties.
Reason for this Review:	Scheduled review
Changes Made:	Yes
Summary:	This is policy details the common features of an acquired brain injury and outlines strategies for staff when supporting people with a new or long term brain injury. It has been reviewed with minor word changes. References and further reading links have been checked and updated.
Relevant Legislation:	• The Care Act 2014 • Care Quality Commission (Registration) Regulations 2009 • Equality Act 2010 • The Health and Social Care Act 2008 (Regulated Activities) Regulations 2014 • Health and Social Care Act 2008 (Registration and Regulated Activities) (Amendment) Regulations 2015 • Mental Capacity Act 2005 • UK GDPR
	• Author: CQC, (2025), Regulation 9: Person-centred care [Online] Available from: https://www.cqc.org.uk/guidance-providers/regulations-enforcement/regulation-9-person-centred-care [Accessed: 26/11/2025] • Author: Brain Injury is Big, (2026), About Brain Injury [Online] Available from: https://www.braininjurygroup.co.uk/living-with-brain-injury/supporting-people-with-brain-injury/support-groups-charities/ [Accessed: 26/11/2025] • Author: National Institute of Health and Care Excellence, (2019), Head Injury: Assessment and early management [Online] Available from: https://www.nice.org.uk/guidance/CG176/chapter/1-Recommendations [Accessed: 26/11/2025] • Author: The Brain Charity, (2025), Brain Injury [Online] Available from: https://www.thebraincharity.org.uk/condition/brain-injury/ [Accessed: 26/11/2025] • Author: Headway, (2025), About Brain Injury [Online] Available from: https://www.headway.org.uk/about-brain-injury [Accessed: 26/11/2025]

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Underpinning Knowledge:	• Author: Andrew Taylor and Richard Morris, (2016), Mental Capacity - Supporting decision making after a brain injury [Online] Available from: https://www.headway.org.uk/media/4108/mental-capacity-supporting-decision-making-after-brain-injury-e-booklet.pdf [Accessed: 26/11/2025] • Author: Brainline, (2009), Interventions for behavioral problems after brain injury [Online] Available from: https://www.brainline.org/article/interventions-behavioral-problems-after-brain-injury#:~:text=%20Interventions%20For%20Behavioral%20Problems%20After%20Brain%20Injury,used%20to%20modify%20behavioral%20problems%20in...%20More%20 [Accessed: 26/11/2025] • Author: Brain Injury E-learning resource, (2017), How to help a person with behavioural difficulties [Online] Available from: https://www.acquiredbraininjury-education.scot.nhs.uk/impact-of-abi/behavioural-difficulties/how-to-help-the-person-with-challenging-behaviours/ [Accessed: 26/11/2025] • Author: Kate Rachel Gould, Jennie Louise Ponsford, Amelia J Hicks, Malcolm Hopwood, Belinda Renison, Timothy J Feeney, (2021), Positive behaviour support for challenging behaviour after acquired brain injury [Online] Available from: https://pubmed.ncbi.nlm.nih.gov/31446844/ [Accessed: 26/11/2025] • Author: Richard Morris, (2018), Caring for someone with a brain injury [Online] Available from: https://www.headway.org.uk/about-brain-injury/individuals/caring/ [Accessed: 26/11/2025] • Author: Headway, (2025), Information library [Online] Available from: https://www.headway.org.uk/about-brain-injury/individuals/information-library/ [Accessed: 26/11/2025]
Suggested Action:	• Encourage sharing the policy through the use of the QCS App
Equality Impact Assessment:	QCS have undertaken an equality analysis during the review of this policy. This statement is a written record that demonstrates that we have shown due regard to the need to eliminate unlawful discrimination, advance equality of opportunity and foster good relations with respect to the characteristics protected by equality law.

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1. Purpose

1.1 To provide IGNIS CARE LIMITED staff guidance and understanding in the complex support needs of the Service User with an acquired brain injury.

1.2

Key Question	Quality Statements
CARING	QSC1: Kindness, compassion and dignity
EFFECTIVE	QSE3: How staff, teams & services work together
EFFECTIVE	QSE4: Supporting people to live healthier lives
RESPONSIVE	QSR1: Person-centred care
SAFE	QSS4: Involving people to manage risks QSS5: Safe environments

1.3 Relevant Legislation

- The Care Act 2014
- Care Quality Commission (Registration) Regulations 2009
- Equality Act 2010
- The Health and Social Care Act 2008 (Regulated Activities) Regulations 2014
- Health and Social Care Act 2008 (Registration and Regulated Activities) (Amendment) Regulations 2015
- Mental Capacity Act 2005
- UK GDPR

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2. Scope

2.1 Roles Affected:

- All Staff

2.2 People Affected:

- People with an acquired brain injury

2.3 Stakeholders Affected:

- Family
- Advocates
- Representatives
- Commissioners
- External health professionals
- Local Authority

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- NHS

3. Objectives

3.1 Ensure staff have an understanding of the types of acquired brain injury.

3.2 Provide guidance and support in how to produce an effective Care Plan and risk assessment to meet the needs of Service Users with an acquired brain injury (ABI).

3.3 Provide an overview of the wide range of complex needs and associated risks for the Service User with an acquired brain injury and provide strategies for staff to support them within a multidisciplinary framework.

3.4 Ensure that staff have an understanding of the behavioural impact of an acquired brain injury and know the practical strategies to effectively support Service Users.

4. Policy

4.1 IGNIS CARE LIMITED accepts that an acquired brain injury (ABI), is defined as 'any trauma to the head which disrupts the function of the brain' (NICE 2007). It may involve the scalp, the skull, the brain or its protective membranes.

A brain injury can leave the Service User with significant support needs such as:

- Physical impairments such as dexterity, posture and mobility
- Communication difficulties including Speech, Language, Swallowing
- Cognitive impairments such as processing information, memory, problem solving, learning, awareness and perception
- Physical and verbal aggression
- Challenging behaviour

This is only a sample of how a brain injury may affect Service Users and the person-centred approach of IGNIS CARE LIMITED will work on the basis that no two brain injuries are the same.

4.2 Traumatic Brain Injury (Headway Charity)

Traumatic brain injury (TBI) is an injury to the brain caused by a trauma to the head (head injury). There are many possible causes, including road traffic accidents, assaults, falls and accidents at home or at work.

The effects of a traumatic brain injury can be wide ranging, and depend on a number of factors such as the type, location and severity of injury.

4.3 Other Forms of Acquired Brain Injury (Headway Charity)

Acquired brain injury covers all situations in which brain injury has occurred since birth, and includes traumatic brain injury as well as tumour, stroke, brain haemorrhage and encephalitis, to name a few (Further information).

The effects are often very similar to those of traumatic brain injury, but there are key differences that make treating and coping with acquired brain injury quite different.

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4.4 The effects and severity of a brain injury vary. This means that each person may experience different challenges and difficulties, depending on the part of the brain affected.

4.5 IGNIS CARE LIMITED staff can view further detailed information and up to date guidance and support tools by accessing:

- Disabilities Trust
- Headway
- The Brain Charity

5. Procedure

5.1 Assessment of Needs

Before IGNIS CARE LIMITED accepts the Service User with an acquired brain injury, the pre-service policies and procedures at IGNIS CARE LIMITED will be followed to ensure that the service can fully meet the identified needs of the individual.

IGNIS CARE LIMITED will work in partnership with MDT partners involved in the current support needs of the individual to ensure enough information is gathered before making an offer of service.

This assessment will take into account the needs of the Service User, current Service Users and compatibility, environmental issues and any required adaptations as well as any staff training that may be required ahead of any new package of support commencing.

5.2 Care Plans and Risk Assessment

The Service User's Care Plan should reflect all the support needs that the person requires from staff as a result of their brain injury.

These may be covered in many parts of the Care Plan such as communication, nutrition and hydration and mobility but also should capture any physical effects, for instance, other health conditions such as Epilepsy, Diabetes and possible hormonal imbalance known as hypopituitarism.

Where the Service User's needs are impacted because of the brain injury specifically, this should be referred to in each of the different Care Plans.

Where there are specific risks, staff should undertake a risk screening on that individual to help identify triggers and support strategies. (The risk screening template can be found in the resource section).

Where possible, IGNIS CARE LIMITED staff should involve the Service User in every aspect of the Care Plan process. Where this is not possible, evidence of best interests should be clear, as set out in 5.4.

Care Plans and risk assessments will be periodically reviewed and audited as part of IGNIS CARE LIMITED quality assurance process to ensure they remain appropriate and effective for the individual Service User.

5.3 External Professionals

Information gathered during the assessment process should highlight the external professionals that may be involved in the Service User's Support or ongoing treatment.

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These can include:

- Specialist case manager - including legal representatives
- Mental health nurse
- Learning disability nurse
- Community mental health services
- Epilepsy nurse
- Social worker
- Care Programme Approach (CPA) co-ordinator
- Criminal justice professionals, e.g. probation, police
- Physiotherapists
- Occupational therapist
- Speech and language therapist

This list is not exhaustive.

Staff should liaise with the relevant professionals and ensure Care Plans contain all contact details available to staff.

Information Sharing

Where information sharing is required to meet the needs of Service Users with external professionals, IGNIS CARE LIMITED UK GDPR and Data Protection policies and procedures will be followed.

5.4 Mental Capacity and Consent

A brain injury can have a significant impact on the Service User's ability to make decisions. To support this, staff will follow IGNIS CARE LIMITED Mental Capacity Act policies and procedures. Accessible information resources, such as easy read formats, will be made available to meet the Service User's unique communication needs, in order to support them to make informed decisions with regards to Support.

In addition, where Service Users lack capacity, the process for best interests will be followed and documented. This will ensure that, where required, Service Users are supported by family, friends, advocacy, legal representatives and involved health professionals as part of the MDT approach at IGNIS CARE LIMITED.

NOTE: The way an (ABI) affects Service Users is very different depending on the injury – some Service Users may lack the capacity to make any decisions at all, even simple day to day choices, such as what to eat or drink, or even what to wear.

It may be that the (ABI) can result in the Service User becoming impulsive and prone to making hasty and irrational decisions regardless of the consequences.

Some Service Users capacity fluctuates for many reasons such as time of day, fatigue, how information is presented and by who. Every effort will be made to ensure support is given in the most appropriate form.

Following the Mental Capacity Act and best interests policies and procedures will result in enabling Service Users to make informed choices and decisions as well as ensuring protections are in place when they are unable to do so for themselves.

Please refer to the Mental Capacity Act (MCA) 2005 Policy and Procedure.

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5.5 Communication Care Plans

IGNIS CARE LIMITED staff understand that communication impairment can be a key factor after brain injury. Headway outline the four main categories of the effects of brain injury.

Any of these can cause communication problems / impairments:

- **Physical**
- **Cognitive**
- **Emotional**
- **Behavioural**

Service Users can often experience more than one communication impairment as a result of a brain injury. These communication difficulties may also impact the Service User's physical ability to speak, rather than the ability to understand and express language. There are two main speech disorders:

- **Dysarthria**
- **Dyspraxia of speech**

IGNIS CARE LIMITED communication policies and procedures will support the development of communication passports and strategies. Staff should ensure that the Service User's Care Plan details the communication needs and any involvement from external professionals such as Speech and Language Therapy.

In addition, IGNIS CARE LIMITED proactively supports the use of Augmentative and Alternative Communication (AAC).

IGNIS CARE LIMITED will work in partnership with specialists in the implementation of strategies, tools and equipment designed to support Service Users with communication impairments.

5.6 Cognition

IGNIS CARE LIMITED Care Plans will include an assessment of any cognitive effects experienced by the Service User as a result of the (ABI).

Staff should be aware of the impact of these on the Service User and the measures and strategies that are in place to reduce any negative impact.

Cognitive effects experienced can include:

- Memory problems
- Language loss (aphasia)
- Impairments in visual perception skills
- Reduced initiative or motivation
- Reduced concentration span
- Reduced information processing ability
- Repetition or perseverance
- Impaired reasoning
- Impaired insight and empathy

5.7 Environmental and Assistive Support

IGNIS CARE LIMITED recognises the importance the environment plays in supporting an individual with a brain injury. IGNIS CARE LIMITED will work in partnership with Health Professionals and others, where appropriate, to introduce assistive technology and

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adaptations into the Service Users environment to improve quality of life and enhance independence.

This support may include:

- Adapting the home and layout
- Increased signage to assist with orientation
- External memory aids such as prompt cards, wall planners, calendars, timers, weekly activity / routine planners, memory notebooks
- Use of mobile apps and other assistive technology

5.8 Physical Impact

Care Plans will detail any physical effects experienced by the Service User such as mobility, fatigue, weakness and sensory impairments as a result of an acquired brain injury.

Care Plans will describe any physical impairments and the impact they have on the quality of the Service Users daily life and the measures and what support is required as a result.

Staff should be aware of some of the physical effects of a brain injury.

5.9 Mental Health and Brain Injury

Service Users with a brain injury and mental health issues may have two separate diagnoses and be treated by different health professionals for each diagnosis.

IGNIS CARE LIMITED recognises that there is an overlap due to the fact that both occur as a result of a dysfunction of the brain.

Regardless of whether a dual diagnosis exists, Service Users who require support from mental health services will have an appropriate Care Plan in place with named professionals who work alongside IGNIS CARE LIMITED staff to offer support.

Care Plans will detail any treatment / support that is in place to assist Service Users in managing any identified mental health conditions.

Where appropriate, advice and referrals for treatment will be sought from the Service User's GP and associated Health Professionals.

Mental Health Crisis Teams

Staff and Service Users who are able will be aware of the contact details of the local mental health crisis teams and these will be documented within a Care Plan.

These teams are usually made up of a number of mental health professionals, such as psychiatrists, mental health nurses, social workers and support workers, to effectively support the Service User who may be experiencing a crisis such as suicidal or psychotic thoughts. Care Plans will detail the circumstances where formal, self-referral and interventions may be sought and how to access them in an emergency.

5.10 Behavioural Effects of an Acquired Brain Injury (ABI)

Behavioural changes in the Service User after a brain injury may be complex and varied.

IGNIS CARE LIMITED will carry out an assessment to identify these behaviours and any support needed as part of a comprehensive Positive Behaviour Support Plan.

Changes may include:

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- Disinhibition
- Impulsiveness
- Obsessive behaviour
- Irritability and aggression
- Apathy and loss of initiative
- Egocentricity

Staff should also follow the Positive Behaviour Support including Challenging Behaviour Policy and Procedure.

5.11 Learning and Development

IGNIS CARE LIMITED recognises that an acquired brain injury will affect all Service Users in very different ways. As part of a training needs analysis, IGNIS CARE LIMITED staff will have relevant specialist training on understanding brain injury and support strategies. In addition, the training needs analysis will identify any further specialist support that is required to meet the needs of individuals and this will be provided.

Where refresher and competencies are also required, these will be added to the annual training plan.

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6. Definitions

6.1 Aphasia

- When a person has difficulty with their language or speech caused by damage to the left side of the brain

6.2 Augmentative Aid to Communication (AAC)

- A range of strategies and tools to help people with speech difficulties
- This may be simple picture boards or sophisticated computer systems

6.3 Brain Tumour

- An abnormal mass of tissue inside the skull, which is caused by cells dividing at an increased speed
- There are two types of brain tumour - malignant and benign

6.4 Stroke

- An emergency condition in which there is a disruption of blood supply to part of the brain, leading to brain injury

6.5 Brain Haemorrhage

- Bleeding in or around the brain either as a result of ruptured aneurysm or following a significant blow to the head
- It is also often called a haemorrhagic stroke or brain bleed

6.6 Encephalitis

- An inflammation of the brain, most often caused by infections

6.7 Traumatic Brain Injury (TBI)

- An injury to the brain caused by a trauma to the head (head injury)

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- There are many possible causes, including road traffic accidents, assaults, falls and accidents at home or at work

7. Key Facts - Professionals

Professionals providing this service should be aware of the following:

- Behavioural changes after brain injury are many and varied. Some appear to be an exaggeration of previous personality characteristics, while others may seem completely out of character for that person
- Brain injury and mental health are often seen and treated as two entirely separate diagnoses, or sometimes confused as being the same thing. This overlap can occur because all cognitive, psychological, emotional and behavioural skills come from the brain, and both brain injury and mental health issues occur because of some dysfunction of the brain
- A brain injury can have a significant impact on a person's ability to make decisions. The cognitive, emotional and behavioural effects of brain injury can make rational decision making very difficult
- The cognitive effects of a brain injury affect the way a person thinks, learns and remembers. Different mental abilities are located in different parts of the brain, so a brain injury can affect some, but not necessarily all, skills such as speed of thought, memory, understanding, concentration, solving problems and using language
- Many people will experience more than one form of communication problem after brain injury, depending on the areas of the brain affected and the severity of the injury
- Memory is easily affected by brain injury because there are several structures within the brain that are involved in memory, and injury to any of these parts can impair memory performance. Staff can support by helping to adapt the persons home, supporting with reminders and helping the person follow a routine
- Staff should be aware of the physical effects of a brain injury and ensure that these needs are highlighted with a plan of support in the Care Plan
- Staff supporting Service Users with a brain injury should have training on understanding brain injury and strategies to support that person

8. Key Facts - People Affected by The Service

People affected by this service should be aware of the following:

- We will ensure that staff supporting you have an understating of your brain injury and how this impacts on you and your daily life
- Staff will follow advice and guidance from external health professionals
- Staff will have training on brain injury when supporting you
- We will involve you in your Care Plan and support you with strategies to help with your day to day living

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Further Reading

United Kingdom Acquired Brain Injury Forum:

https://ukabif.org.uk/

List of self-help websites:

https://www.getselfhelp.co.uk/

Brain and Spine Foundation:

https://www.brainandspine.org.uk/

Different Strokes - Stroke Charity:

https://differentstrokes.co.uk/

Stroke Association:

https://www.stroke.org.uk/

Brain Tumour Charity:

https://www.thebraintumourcharity.org/

Communication Matters:

https://www.communicationmatters.org.uk/wp-content/uploads/2019/02/What-is-AAC.pdf

The community mental health framework for adults and older adults

https://www.england.nhs.uk/publication/the-community-mental-health-framework-for-adults-and-older-adults/

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Outstanding Practice

To be "outstanding" in this policy area you could provide evidence that:

- Staff possess a comprehensive understanding of brain injuries and their effects on Service Users
- There is evidence of staff adapting home environments and Care Plans to support daily living for Service Users with brain injuries
- There is evidence that staff with specialized skills and knowledge support Service Users with brain injuries, offering a service that reflects positive risk taking and enables people to live the life they choose
- Care Plans reflect the advice of external health professionals and address various needs of Service Users
- Best practices and legislation regarding acquired brain injuries are followed, with ongoing support and development from key forums and support groups

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- The wide understanding of the policy is enabled by proactive use of the QCS App

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